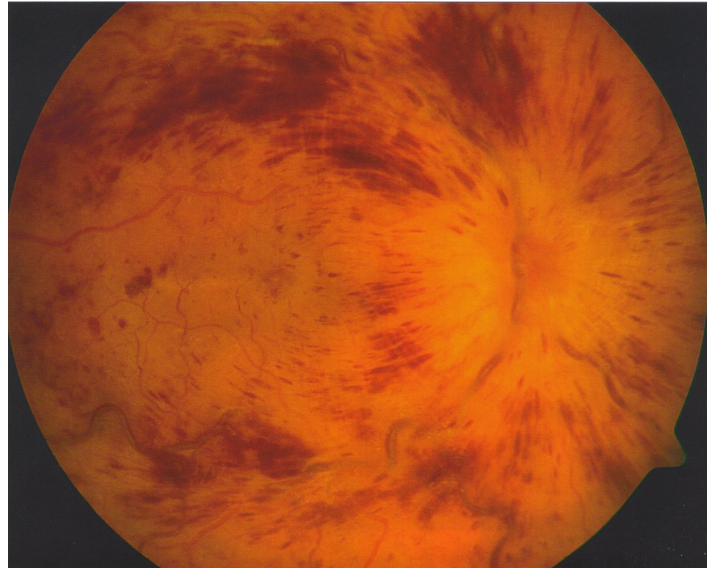


CRVO: the neovascular pathway

Clinical M3-M5 | Focused Type B notes

Oedema and ischaemia answer different questions

A diffuse venous haemorrhagic pattern



Diffuse haemorrhages and venous congestion fit CRVO. Ischaemic severity requires function, pupils and perfusion assessment; a colour photo alone is insufficient. Supplied M3 Anki, media 133.

Two major pathways

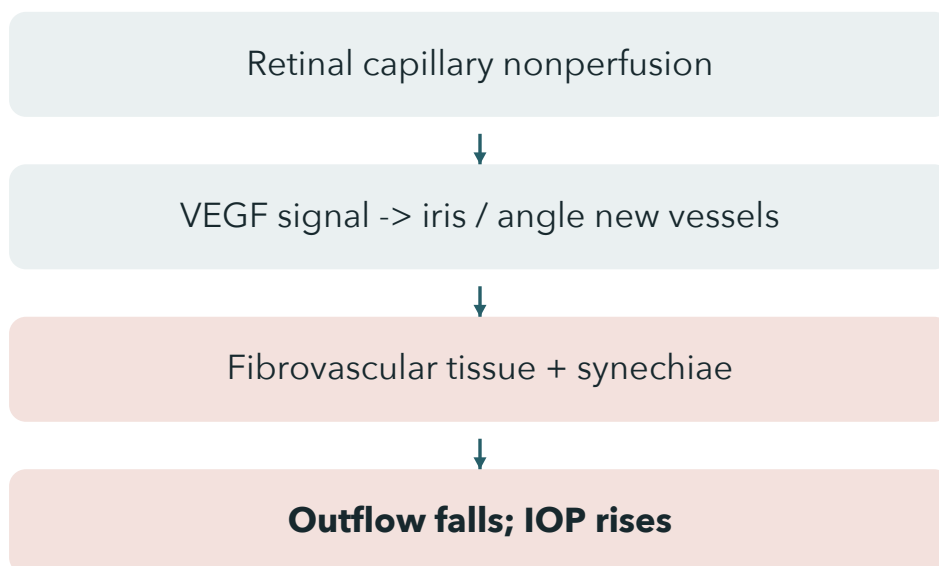
Macular oedema	Retinal ischaemia
OCT fluid may reduce central acuity.	RAPD, poor function and capillary nonperfusion indicate risk.
Anti-VEGF can improve the fluid.	Monitor iris and angle new vessels.
A dry OCT tracks oedema response.	A dry OCT does not prove reperfusion.

Do not let treatment mask risk

Anti-VEGF can suppress visible new vessels temporarily. Eyes with ischaemic CRVO require close anterior-segment surveillance, including after injections are stopped.

New iris vessels are an action trigger

Posterior ischaemia can obstruct anterior outflow



Original route from retinal nonperfusion to iris/angle vessels, synechial closure and elevated IOP.

Respond to iris or angle neovascularisation

- 1 Arrange prompt retina/glaucoma assessment and appropriate pressure control.
- 2 Use anti-VEGF as an adjunct for rapid vessel regression.
- 3 Deliver adequate PRP to reduce the retinal ischaemic drive; continue surveillance.

Prophylaxis is selective

Do not automatically laser every CRVO solely because haemorrhages look dramatic. PRP is usually triggered by neovascular complications; prophylaxis may be considered in selected high-risk situations when reliable follow-up is not possible.